Adverse Events Classification

Introduction

All AE severity grades follow this generic framework. If there are no severity scales specified for an AE, this generic scale should be used.

Grade 1	Grade 2	Grade 3	Grade 4	Grade 5
Mild	**Moderate**	**Severe**	**Life threatening**	**Death**
Mild; asymptomatic or mild symptoms; clinical or diagnostic observations only; no change in baseline age-appropriate behavior*; no change in baseline care or monitoring indicated	Moderate; resulting in minor changes of baseline age-appropriate behavior*; requiring minor changes in baseline care or monitoring***	Severe; resulting in major changes of baseline age-appropriate behavior* or non-life-threatening changes in basal physiological processes**; requiring major change in baseline care or monitoring****	Life-threatening; Resulting in life-threatening changes in basal physiological processes**; requiring urgent major change in baseline care	Death related to AE

*Age-appropriate behavior refers to oral feeding behavior, voluntary movements and activity, crying pattern, social interactions and perception of pain.
**Basal physiological processes refer to oxygenation, ventilation, tissue perfusion, metabolic stability and organ functioning.
***Minor care changes constitute brief, local, non-invasive or symptomatic treatments
****Major care changes constitute surgery, addition of long-term treatment, upscaling care level
If the different factors of this scale result in conflicting severity grades, the highest grade should be reported.
A single dash (-) indicates a severity grade is not available. Not all grades are appropriate for all AEs.

S.No	Adverse Events	Grade 1 (Mild)	Grade 2 (Moderate)	Grade 3 (Severe)	Grade 4 (Life-threatening)	Grade 5 (Death)
Neurological						
1.	**Neonatal Convulsion** Definition C154952 | 10028932: Sudden, involuntary, rapid, rhythmic or stereotyped skeletal muscular contractions in a newborn.	Single, self-limited suspected seizure, no treatment	Suspected seizures controlled with 1 anti-seizure drug (no recurrence within 3 days after treatment)	Suspected seizures uncontrolled with 1 anti-seizure drug (recurrence within 3 days after treatment or requiring 2 or more anti-seizure drugs)	Suspected seizures with life-threatening consequences (e.g. need for ventilation); suspected status epilepticus* despite multiple anti-seizure drugs	Death related to suspected seizures.
	***** >30 min duration of convulsions within any 60-minute period					

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2.	**Neonatal Epileptic Seizure** Definition C154934 | 10082068 (v22.0): An EEG-confirmed paroxysmal surge of electrical activity in the brain that may result in physical or behavioral changes in a neonate.	Single, self-limited EEG-proven seizure*, no treatment	EEG-proven seizures controlled with 1 anti-seizure drug (no recurrence within 3 days after treatment)	EEG-proven seizures uncontrolled with 1 anti-seizure drug (recurrence within 3 days after treatment or requiring 2 or more anti-seizure drugs)	EEG-proven seizures with life-threatening consequences (e.g., need for ventilation); status epilepticus** despite multiple anti-seizure drugs	Death related to seizures*
	***** Electrographic confirmation required: can be electro-clinical or electrographic only ****** >30 min of seizure activity within any 60-minute period					
3.	**Neonatal Intraventricular Hemorrhage (IVH)** Definition C154937 | 10022844: Bleeding into the lateral cerebral ventricles in a newborn infant.	Asymptomatic hemorrhage confined to the germinal matrix; minimal hemorrhage within the ventricle (<10%) on parasagittal view	Moderate hemorrhage occupying ≤50% of ventricle volume (<50%) without ventricular dilatation > 4 mm above the 97th percentile	Hemorrhage occupying >50% of the ventricle volume; ventricular dilatation > 4 mm above the 97th percentile; parenchymal venous infarction; requiring temporizing neurosurgical procedure (drain, shunt, or reservoir)	Hemorrhage with parenchymal venous infarction; resulting in life-threatening consequences (e.g., refractory seizures, hypotension, respiratory depression); requiring urgent stabilization or surgical decompression	Death
4.	**Hypoxic Ischemic Encephalopathy (HIE)** (Definition C154945 | 10070512): Injury to the central nervous system that occurs when there is insufficient delivery of oxygen to all or part of the brain.	Mild transient clinical signs of encephalopathy resulting from a perinatal asphyxia event; meeting criteria for observation and additional care	Moderate clinical signs of encephalopathy resulting from a perinatal asphyxia event necessitating therapeutic hypothermia	Severe clinical signs of encephalopathy resulting from a perinatal asphyxia event	An event clinically classified as perinatal asphyxia, but not resulting in encephalopathy with life-threatening consequences (e.g. respiratory depression, refractory seizures)	Death
	**see Sarnat stages for further guidance* on mild, moderate and severe clinical signs of encephalopathy					

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5.	**Periventricular Leukomalacia (PVL)** (Definition C154923 | 10052594): A form of cerebral white matter injury usually seen in preterm infants that is characterized by necrotic degeneration or gliosis of white matter adjacent to the cerebral ventricles that may evolve into focal cysts.	Transient periventricular echo densities persisting > 7 days and resolving completely	Transient periventricular echo densities evolving into small localized fronto-parietal cysts or persistent diffuse echo densities.	Periventricular echo densities, evolving into extensive cystic periventricular lesions; or densities extending into the deep white matter		
6.	**Infant Irritability** (Definition C154939 | 10082189): Crying easily, difficult to console.	Mild, self-limiting irritability, not affecting feeding and sleeping	Moderate irritability, minor changes in feeding and sleeping behavior, requiring minor additional care (e.g. occasional analgesics)	Severe irritability, major changes in feeding behavior, requiring support other than oral feeding; requiring long-term medical treatment (e.g. sedatives)	Life-threatening irritability, with loss of autonomic control of temperature or heart rate; requiring urgent care change	
7.	**Infant Sedation** (Definition C154926 | 10082187): A state of a lowered level of consciousness.	Mild, self-limiting or transient sedation or sleepiness; transient loss of interactions; not requiring care change	Moderate sedation; limiting interactions and minor changes in feeding behavior; requiring increased monitoring	Severe sedation, major changes in feeding behavior; requiring support other than oral feeding (e.g. tube feeding)	Life-threatening sedation with cardiorespiratory instability; requiring urgent care change	
8.	**Hydrocephalus/Ventriculomegaly:** A disorder characterized by dilatation of the ventricular system	-	Arrested mild ventriculomegaly NOT needing medical therapy or placement of shunt.	Ventriculomegaly or hydrocephalus needing medical therapy or placement of shunt, increased risk of adverse neurological outcome	Hydrocephalus causing cardiorespiratory compromise	Death
9.	**Meningitis/Encephalitis:** A disorder characterized by inflammation of meninges and/or brain matter caused by an infective agent	-	-	Meningitis/encephalitis without shock or end-organ failure requiring antibiotic therapy, causing prolongation of hospital stay or increased risk of adverse neurological outcome	Accompanied by shock or end-organ failure. Life-threatening consequences; urgent surgical intervention indicated	Death

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10.	**Brain abscess:** A disorder characterized by the presence of a bacterial abscess in brain parenchyma	-	-	Brain abscess needing treatment with antibiotics or surgical drainage resulting in increased probability of adverse neurological outcome.	Brain abscess resulting in cardiorespiratory compromise.	Death
11.	**Stroke:** A disorder characterized by arterial ischemic or hemorrhagic infarction of brain parenchyma	-	-	Stroke in any brain area resulting in increased probability of adverse neurological outcome	Stroke in any brain area resulting in cardiorespiratory compromise	Death
12.	**Porencephalic cyst**	-	-	Porencephalic cyst resulting in increased probability of adverse neurological outcome	-	Death
Cardiovascular						
13.	**Neonatal Hypotension** (Definition C154930 | 10049223): Abnormally low blood pressure, which is usually symptomatic.	Transient (<60/min), asymptomatic hypotension; not affecting perfusion; not requiring intervention	Persistent (>60/min) hypotension; not affecting perfusion; requiring minor care changes (e.g. additional fluids)	Persistent hypotension affecting perfusion; requiring major care change (e.g. vaso-active drugs or hydrocortisone).	Life-threatening consequences (e.g. shock, organ failure).	Death
14.	**Neonatal Hypertension** (Definition C154929 | 10049781): Abnormal high blood pressure.	Systolic or diastolic BP>90th percentile but <95th percentile; self-limiting.	Persistent or recurrent hypertension, with systolic or diastolic BP between the 95th percentile and the 99th percentile.	Persistent or recurrent hypertension, with systolic or diastolic BP above the 99th percentile; need for antihypertensive medication.	Life-threatening consequences (e.g. malignant hypertension: shock, cardiac failure or neonatal encephalopathy).	Death
15.	**Neonatal Sinus Tachycardia** (Definition C154932 | 10082191): An abnormally high heart rate for age.	Brief episodes of tachycardia, self-limiting; no change in age-appropriate behavior; no care changes	Persistent tachycardia; no change in age-appropriate behavior; requiring minor care changes (e.g. concomitant medication changed)	Persistent tachycardia; resulting in non-life-threatening hemodynamic compromise; requiring major care changes (e.g. new medication or intervention)	Life-threatening consequences; requiring urgent major care changes	Death
16.	**Neonatal Sinus Bradycardia** (Definition C154933 | 10082188):	Brief, self-limiting episodes of	Persistent bradycardia, no change in age-appropriate behavior; requiring minor	Persistent bradycardia, resulting in non-life-threatening	Life-threatening consequences (e.g. shock); requiring	Death

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	An abnormally low heart rate for age.	bradycardia, no care changes	care changes (e.g. concomitant medication adapted, intermittent increase FiO_2)	hemodynamic compromise; requiring major care changes (e.g. new medication or intervention)	urgent major care changes	
17.	**Neonatal Tachyarrhythmia** (Definition C154948 | 10082055): Non-sinus rhythm with an abnormally high heart rate for age.	Brief, self-limiting episodes of asymptomatic tachyarrhythmia (e.g. extrasystolic beats); no care changes	No change in age-appropriate behavior; requiring minor care changes (e.g. increased monitoring)	Resulting in non-life-threatening hemodynamic compromise or changes in age-appropriate behavior; requiring major care changes (e.g. new medication or intervention)	Life-threatening consequences (e.g. shock); requiring urgent major care changes	Death
18.	**Neonatal Bradyarrhythmia** (Definition C154949 | 10082054): Non-sinus rhythm with an abnormally low heart rate for age.	Brief, self-limiting episodes of asymptomatic bradyarrhythmia; no care changes	No change in age-appropriate behavior; requiring minor care changes (e.g. increased monitoring)	Resulting in non-life-threatening hemodynamic compromise; requiring major care changes (e.g. new medication or intervention)	Life-threatening consequences (e.g. shock); requiring urgent major care changes	Death
19.	**Shock:** A disorder characterized by inability of circulatory system to meet metabolic requirements of tissues	-	Hypotension or decreased organ perfusion lasting for <6 hours needing volume expander and resolving without consequence	Hypotension or decreased organ perfusion needing /vasopressor(s)/ inotropes	Catecholamine-resistant shock	Death
20.	**Patent Ductus Arteriosus (PDA)**	PDA NOT needing treatment or resulting in prolongation of respiratory support/hospital stay	PDA needing medical treatment but NOT cardiac failure	PDA needing surgical treatment or resulting in cardiac failure	-	Death
21.	**Myocarditis:** A disorder characterized by inflammation of the myocardium diagnosed by elevation of cardiac enzymes or on cardiac biopsy	-	-	Need for medical therapy including decongestive therapy	Cardiorespiratory compromise resulting in need of artificial cardiac or respiratory support	Death
22.	**Thromboembolism:** A disorder characterized by thromboembolic phenomenon involving a vascular support	Asymptomatic, NOT needing treatment	Causing gangrene of distal phalanx	Causing gangrene extending > 1 phalanx of any digit or metacarpals/metatarsals	Causing gangrene including long bones of limbs or any internal organs like	-

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					brain, kidneys or lungs	
Respiratory						
23.	**Infantile Apnea** (Definition C154938 | 10077322): Cessation of airflow.	Self-limiting apnea	Apnea responsive to stimulation or intermittent FiO2-increase.	Apnea requiring stimulation and sustained FiO2 increase; requiring non-invasive ventilation; reoccurrences requiring start of or relevant increase in dose of respiratory stimulants or other major care changes	Life-threatening respiratory and/or hemodynamic compromise; (semi) urgent intubation required	Death
24.	**Neonatal Respiratory Insufficiency** (Definition C154951 | 10082057): Significant impairment of gas exchange resulting in compensatory breathing efforts and eventually hypoxia and/or hypercarbia.	Clinical evidence of mildly increased respiratory distress (e.g. increased work of breathing) without significant deterioration in gas exchange; no change in baseline functioning; no care changes required	Clinical evidence of increased respiratory distress without significant deterioration in gas exchange (increase in pCO2 or decrease in oxygenation); corrected by minor adjustments in current ventilator support, supplemental oxygen or non-invasive support	Clinical evidence of increased respiratory distress with relevant deterioration in gas exchange (increase in pCO2 or decrease in oxygenation); requiring major care change (e.g. start invasive support)	Life-threatening respiratory and/or hemodynamic compromise; requiring urgent care change (e.g. urgent intubation)	Death
	*Consider this adverse event for any patient whose respiratory condition deteriorates from baseline. If any specific diagnosis is identified: use diagnosis-specific scale (e.g. RDS, pulmonary hemorrhage)					
25.	**Neonatal Respiratory Distress Syndrome (RDS)** (Definition C154950 | 10028974): Progressive alveolar atelectasis from birth due to an abnormality of synthesis, function or metabolism of surfactant, characterized by respiratory failure and an abnormal chest radiograph showing diffuse reticulogranular densities and air bronchograms.	Radiological evidence without clinical signs; clinical evidence of mildly increased respiratory distress (e.g. increased work of breathing) without significant deterioration in gas exchange; no supportive care required	Clinical evidence of increased respiratory distress without significant deterioration in gas exchange (increase in pCO2 or decrease in oxygenation); corrected by minor adjustments in current ventilator support, supplemental oxygen or non-invasive support	Clinical evidence of increased respiratory distress with relevant deterioration in gas exchange (increase in pCO2 or decrease in oxygenation); requiring major care change (e.g. start invasive support)	Life-threatening respiratory and/or hemodynamic compromise; requiring urgent care change (e.g. urgent intubation)	Death

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26.	**Neonatal Pulmonary Hemorrhage** (Definition C154936 | 10082199): Bleeding in the respiratory tract of a neonate.	Limited hemorrhagic secretion in ET tube	Hemorrhagic secretion in ET tube; without relevant increase in PCO2 or decrease in oxygenation; requiring minor changes in care (e.g. increase PEEP)	Hemorrhagic secretion in ET tube; with relevant increase in PCO2 or decrease in oxygenation; requiring major change in ventilator support or transfusion	Life-threatening respiratory and/or hemodynamic compromise	Death
27.	**Persistent Pulmonary Hypertension of the Newborn (PPHN)** (Definition C154940 | 10082195): Elevated pulmonary vascular pressure in a neonate.	Technical evidence of increased RV pressures; no clinical symptoms	Increased estimated RV-pressures; with moderate clinical symptoms; oxygenation index <25; minor care changes required	Increased estimated RV-pressures; with severe clinical symptoms; oxygenation index >25; major care changes required (e.g. iNO)	Life-threatening respiratory and/or hemodynamic compromise; ECMO required; oxygenation index >40	Death
28.	**Neonatal Pneumothorax** (Definition C154920 | 10082056): A collection of air or other gas between the visceral and parietal pleura.	Radiological evidence of pneumothorax; no clinical signs; no care change required	Radiological evidence of pneumothorax; minor clinical signs; minor care change required (e.g. oxygen, increased monitoring)	Radiological evidence of pneumothorax; with clinical signs; major care change required (e.g. chest drainage)	Life-threatening respiratory and/or hemodynamic compromise (e.g. tension pneumothorax); urgent major care change required	Death
29.	**Bronchopulmonary Dysplasia** (Definition C154919 | 10006475): A chronic lung disorder associated with pulmonary maldevelopment, scarring, and/or inflammation that develops in preterm neonates. The condition is defined based on treatment with supplemental oxygen for at least 28 days adjusted for the degree of prematurity.	Supplemental oxygen at 28 days AND Breathing room air at 36 weeks PMA in infants born <32 weeks' gestation; breathing room air by 56 days postnatal age in infants born >32 weeks' gestation; breathing room air at discharge	Supplemental oxygen at 28 days AND need for 22-30% oxygen or positive pressure at 36 weeks PMA in infants born <32 weeks' gestation; need for 22-30% oxygen by 56 days postnatal age in infants born >32 weeks' gestation; need for 22-30% oxygen at discharge	Supplemental oxygen at 28 days AND need for >30% oxygen at 36 weeks PMA in infants born <32 weeks' gestation; need for >30% oxygen and/or positive pressure by 56 days postnatal age in infants born >32 weeks' gestation; need for >30% oxygen and/or positive pressure at discharge	Supplemental oxygen at 28 days AND need for >30% oxygen AND positive pressure at 36 weeks PMA in infants born <32 weeks' gestation; need for >30% oxygen AND positive pressure by 56 days postnatal age in infants born >32 weeks' gestation; need for >30%	Death

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					oxygen AND positive pressure at discharge.	
	**For conversion of oxygen administered by different modalities to FiO2 see for example http://nicutools.org/MediCalcs/ActualO2.php3*					
30.	**Ventilator-associated Pneumonia:** A disorder characterized by appearance of new infiltrates or shadows in chest x-ray in a neonate receiving respiratory support	-	-	New infiltrates or shadows in chest x-ray along with increased oxygen or ventilation requirements resulting in prolongation of assisted ventilation	New infiltrates or shadows in chest x-ray along with increased oxygen or ventilation requirements resulting in need of high frequency ventilation due to hypoxia or hypercarbia	Death
31.	**Lung collapse:** A disorder characterized by collapse/atelectasis of lung or its segment/lobe	-	Lung collapse involving segments/lobe NOT causing new symptoms, or resulting in increase in oxygen/ventilator requirement or increased duration of ventilator requirement or prolongation of hospital stay.	Lung collapse with increased oxygen or ventilation requirements resulting in prolongation of assisted ventilation	Lung collapse needing high frequency ventilation due to hypoxia or hypercarbia.	Death
32.	**Subglottic stenosis:** A disorder characterized by stenosis in the subglottic airway	Transient (<24 hours) increase in oxygen requirement, respiratory difficulty or stridor, or needing non-invasive respiratory support or inhalation drug therapy	Subglottic stenosis resulting in extubation failure	Significant respiratory distress resulting in more than 2 extubation failures or need for systemic steroid therapy or tracheostomy	-	-
33.	**Pleural effusion:** A disorder characterized by the presence of free fluid in the pleural cavity	-	Minimal fluid in the pleural cavity NOT causing any increase in oxygen or ventilator requirement or drainage.	Free fluid in the pleural cavity NOT causing an increase in oxygen or ventilator requirement or drainage or need for high-frequency ventilation.	Fluid in pleural cavity resulting in circulatory collapse	Death
34.	**Aspiration syndromes:** A disorder characterized by aspiration of milk, blood, vomitus, or any other fluid into the lungs	-	New infiltrates or shadows in chest X-ray along with need for oxygen therapy by hood or nasal cannula	New infiltrates or shadows in chest X-ray requiring invasive or non-invasive ventilation or increased	New infiltrates or shadows in chest X-ray along with increased oxygen or ventilation	Death

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				ventilation requirements resulting in prolongation of assisted ventilation	requirements resulting in need of high frequency ventilation due to hypoxia or hypercarbia	
Gastro-intestinal						
35.	**Necrotizing Enterocolitis (NEC)** (Definition C154922 | 10052818): A disease of neonates in which there is extensive mucosal ulceration, pseudomembrane formation, submucosal hemorrhage, and necrosis.	-	-	NEC confirmed; major care change indicated (e.g., NPO, antibiotics, non-urgent surgery)	Bowel perforation (pneumoperitoneum) (Bell IIIB); shock, DIC, combined respiratory and metabolic acidosis (Bell IIIA); urgent major care change indicated	Death
	**If NEC is not confirmed (Bell stages 1): please record severity of individual symptoms (e.g. feeding intolerance).*					
36.	**Neonatal Diarrhea** (Definition C154953 | 10012731): Watery bowel movements.	Increase of 2-4 stools per day over baseline; mild increase in ostomy output compared to baseline	Increase of 4-6 stools per day over baseline; moderate increase in ostomy output compared to baseline	Increase of >=7 stools per day over baseline; severe increase in ostomy output compared to baseline; signs of dehydration	Life-threatening consequences (e.g. severe dehydration)	Death
37.	**Infantile Vomiting** (Definition C154942 | 10075315): Expulsion of the contents of the stomach through the mouth.	Increase in vomiting over baseline, self-limiting.	Persistent increase in vomiting over baseline; no dehydration; minor changes in feeding support indicated	Persistent increase in vomiting over baseline; signs of dehydration; major changes in feeding support indicated (e.g. change to TPN or gavage)	Life-threatening consequences (e.g. severe dehydration)	Death

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38.	**Feeding Intolerance** (Definition C154944 | 10076042): Inability to achieve a full feeding volume.	Mild feeding intolerance (e.g. increased gastric residual volume or abdominal distension); without discomfort; no change in care indicated	Moderate feeding intolerance; resulting in minor discomfort or alteration of drinking behavior; minor care changes indicated (e.g. feeds withheld)	Severe feeding intolerance; requiring major changes in feeding support indicated (e.g. change to TPN or gavage)	Life-threatening consequences	Death
39.	**Neonatal Gastrointestinal Bleeding** (Definition C154943 | 10082192): Hemorrhage originating at any site located within the gastrointestinal tract.	Mild, self-limiting bleeding; no change in care required	Moderate bleeding; minor change in care or monitoring required	Severe bleeding; non-life-threatening hemodynamic consequences; major care change required (e.g. invasive intervention, transfusion, long-term medical treatment)	Life-threatening consequences (e.g. hemorrhagic shock); urgent major care change required	Death
40.	**Neonatal Spontaneous Intestinal Perforation (SIP)** (Definition C154924 | 10082196): A perforation in the gastrointestinal tract of a newborn with no demonstrable cause.	-	-	Presence of SIP, non-urgent medical stabilization and surgical intervention indicated	Life-threatening consequences (e.g., shock, organ failure); urgent intervention indicated	Death
41.	**Neonatal Constipation** (Definition C154946 | 10082190): Irregular and infrequent or difficult evacuation of the bowels.	Reduced number of stools or hard stools; no apparent discomfort	Reduced number of stools or hard stools; apparent discomfort; minor care changes indicated	Reduced number of stools or hard stools; signs of obstruction; apparent discomfort or affecting feeding; major care changes indicated (e.g. long term medical treatment, enema)	Life-threatening consequences	Death
42.	**Gastroenteritis:** A disorder characterized by inflammation of	Gastroenteritis without dehydration	Gastroenteritis with moderate dehydration	Gastroenteritis with severe dehydration	Gastroenteritis with shock	Death

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	the gastrointestinal tract manifesting as loose motions, vomiting, fever and dehydration. If gastroenteritis is a manifestation of sepsis – include it under the heading of sepsis, not under gastroenteritis.					
43.	**Peritonitis:** A disorder characterized by inflammation of peritoneal membranes	-	-	Peritonitis unaccompanied by perforation of gut	Peritonitis accompanied by perforation of gut	Death
44.	**Neonatal cholestasis**	-	Neonatal hepatitis or cholestasis which resolves spontaneously or with medical therapy	Neonatal hepatitis or cholestasis which does NOT resolve with treatment or is due to an untreatable metabolic condition or needs liver transplantation	Neonatal hepatitis or cholestasis which progresses to liver failure	Death
45.	**Hepatic failure:** A disorder characterized by failure of liver functions manifesting as one or more of the following: decreased serum albumin, coagulopathy, raised liver enzymes, raised serum bilirubin, ascites	-	-	Hepatic failure without encephalopathy	Hepatic failure with encephalopathy	Death
46.	**Liver abscess:** A disorder characterized by the presence of an infective abscess in the liver	-	-	Uncomplicated liver abscess	Complicated liver abscess (e.g. rupture, peritonitis)	Death
Infectious						
47.	**Neonatal Culture Negative Sepsis** (Definition C154928 | 10082059): A systemic inflammatory response without identifiable cause.	-	Suspected sepsis with mild or ambiguous signs; antibiotics initiated	Suspected sepsis with severe signs (e.g. fever, grunting); support treatment escalated or initiated; care change required	Life-threatening consequences (e.g. state of shock, DIC); urgent major care change required	Death
48.	**Neonatal Culture Positive Sepsis** (Definition C154927 | 10082058): A systemic inflammatory response to an infection.	Blood culture positive; no care change indicated (e.g. contamination suspected)	Blood culture positive with mild or ambiguous signs; antibiotics initiated	Blood culture positive with severe signs; support treatment escalated or initiated; care change required	Life-threatening consequences (e.g. state of shock, DIC); urgent major care change required	Death
49.	**Neonatal Rash** (Definition C154935 | 10037871): An eruption	Localized rash	Diffuse rash or target lesions	Diffuse rash and vesicles or limited	Extensive or generalized bullous	Death

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	in the skin which affects its appearance and/or texture			number of bullae or superficial ulcerations of mucous membrane limited to one site	lesions or ulceration of mucous membrane involving 2 or more distinct mucosal sites or Stevens-Johnson syndrome or toxic epidermal necrosis	
50.	**Neonatal Administration Site Complication** (Definition C154947 | 10082193): Local irritation or complication at the administration site of a drug.	Painless edema	Erythema with associated symptoms (e.g., edema, pain, induration, phlebitis)	Ulceration or necrosis; severe tissue damage; operative intervention indicated	Life-threatening consequences; urgent intervention indicated	Death
51.	**Neonatal Fever** (Definition C154941 | 10016562): Elevation of body temperature above normal due to the production of more heat than the body is able to dissipate.	Isolated, not sustained fever, no change in baseline age-appropriate behavior	Fever, minor change in baseline age-appropriate behavior; minor changes in care (e.g., environmental settings, symptomatic treatment)	Fever, major change in baseline age-appropriate behavior and without resolution to symptomatic treatment.	Life-threatening consequences.	Death
52.	**Bone-joint infection:** A disorder characterized by infection of bone or joint space	-	-	Bone-joint infection without shock or end-organ failure requiring antibiotic therapy and causing prolongation of hospital stay.	Accompanied by cardiorespiratory compromise	Death
Hematological						
53.	**Neonatal Coagulation Disorder** (Definition C154921 | 10009732): A condition of abnormal blood clotting or bleeding.	Minor biochemical coagulation abnormalities without clinical signs; no care changes indicated	Biochemical coagulation abnormalities with clinical signs; requiring intervention	Biochemical or clinical coagulation abnormalities; requiring increased monitoring	Life-threatening consequences (e.g. severe pulmonary embolism, limb ischemia, hemorrhagic shock, DIC); requiring urgent major care changes	Death
54.	**Neutropenia:** A disorder characterized by a decrease in the number of neutrophils in the peripheral blood film	Asymptomatic and NOT associated with systemic infection, NOT needing any therapeutic intervention	Asymptomatic and NOT associated with systemic infection, needing therapeutic intervention	Associated with systemic infection	-	-

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55.	**Thrombocytopenia:** A disorder characterized by a decrease in the number of platelets (less than 15,000/microL) in peripheral blood	NOT associated with active bleeding and above the threshold for platelet transfusion	NOT associated with active bleeding but needing platelet transfusion	Associated with bleeding or platelet count less than 20,000	Associated with intracranial bleeding	Death
56.	**Coagulopathy/DIC:** A disorder characterized by a state of altered coagulation status due to abnormal activation of the coagulation cascade	-	-	Laboratory evidence, but no significant clinical bleeding	Significant clinical bleeding along with laboratory evidence	Death
57.	**Anemia:** A disorder characterized by decreased hemoglobin in blood	Asymptomatic anemia NOT needing blood transfusion	-	Anemia needing blood or exchange transfusion	-	-
58.	**Polycythemia:** A disorder characterized by increased hemoglobin in blood as per gestation and postnatal gestation norms	Asymptomatic, polycythemia NOT needing treatment	Symptomatic polycythemia needing treatment with fluid relaxation	Symptomatic polycythemia partial exchange transfusion or associated with complications	-	-
Metabolic						
59.	**Hyperbilirubinemia:** A disorder characterized by an increase in indirect bilirubin levels in blood	Hyperbilirubinemia without need of therapy	Hyperbilirubinemia needing treatment with phototherapy	Hyperbilirubinemia needing treatment with blood exchange transfusion	Hyperbilirubinemia with acute bilirubin encephalopathy	Death
60.	**Hypernatremia:** A disorder characterized by an increase in concentration of sodium ions in blood	Serum sodium 146-150 mEq/L	Serum sodium 151-160 mEq/L	Serum sodium 161-170 mEq/L	Serum sodium >170 mEq/L or accompanied by clinical features of seizures or altered consciousness	Death
61.	**Hyperkalemia:** A disorder characterized by an increase in the concentration of potassium ions in blood	Serum potassium 5.5-6.5 mEq/L	Serum potassium 6.5-8.0 mEq/L	Serum potassium >8 mEq/L	-	Death
62.	**Hyponatremia:** A disorder characterized by a decrease in the concentration of sodium ions in blood	Serum sodium 130-134 mEq/L	Serum sodium 120-129 mEq/L	Serum sodium 110-119 mEq/L	Serum sodium <110 mEq/L or accompanied by clinical features of seizures or altered consciousness	Death
63.	**Hypoglycemia:** A disorder characterized by blood glucose concentration less than 40 mg/dL	Blood glucose 20-40 mg/dL: asymptomatic, treated single episode, supervised feeds.	More than one episode of blood glucose 20-40 mg/dL or any blood glucose <20 or with symptoms other than	Seizures or need for intravenous glucose infusion @ ≥12 mg/kg/min or persisting for > 7 days.	-	-

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			seizures or need for intravenous glucose infusion up to 12 mg/kg/min.			
64.	**Hyperglycemia:** A disorder characterized by blood glucose concentration greater than 150 mg/dL	NOT needing treatment with insulin	Need for treatment with insulin	-	-	-
Thermoregulation						
65.	**Hypothermia:** A disorder characterized by axillary temperature less than 36.5 °C	Axillary temperature 36.0°C-36.4°C	Axillary temperature 32.0°C-35.9°C	Axillary temperature <32.0°C	-	-
66.	Hyperthermia: A disorder characterized by axillary temperature more than 37.5 °C	Axillary temperature 37.6°C-38.0°C	Axillary temperature 38.1°C-40.0°C	Axillary temperature >40.0°C	-	Death
Sensory						
67.	**Retinopathy of Prematurity (ROP)** (Definition C154925 | 10038933): A retinal condition of very immature infants that may be characterized by a non-vascularized retina that may lead to neovascularization, scarring, retinal detachment, and blindness.	Zone 2 ICROP stage 2 with or without plus disease; zone 3 any ICROP stage; no care changes indicated	Type 2 pre-threshold ROP (zone 1 ICROP stage 1 or 2 without plus disease; zone 2 ICROP stage 3 without plus disease); requiring more frequent ophthalmic monitoring.	Type 1 pre-threshold ROP (zone 1 any stage with plus disease; zone 1 ICROP stage 3 without plus disease; zone 2 ICROP stage 2 or 3 with plus disease); threshold ROP; requiring major care changes (e.g. laser intervention, intravitreal anti-VEGF or operative management)	Unilateral retinal detachment	Blindness (bilateral retinal detachment,...)
68.	**Sensorineural hearing loss (SNHL):** A disorder characterized by SNHL	SNHL NOT needing any hearing aid	SNHL with one ear spared of need for a hearing aid	SNHL needing hearing aid in both ears	-	-
69.	**Severe nasal trauma**	Self-healing nasal trauma NOT causing permanent disfigurement.	-	Nasal trauma which causes severe and permanent disfigurement and which needs surgical correction	-	-
Renal						

S.No	Adverse Events	Grade 1 (Mild)	Grade 2 (Moderate)	Grade 3 (Severe)	Grade 4 (Life-threatening)	Grade 5 (Death)
70.	**Acute Kidney Injury/Acute Renal Failure (AKI/ARF):** A disorder characterized by sudden impairment in kidney function resulting in inability to maintain fluid, electrolyte, and waste homeostasis	-	Serum creatinine increase of >=0.3 within 48 h OR 1.5-1.9 times the lowest previous value within 7 days; Urine output: <0.5 ml/kg/h for 6-12 h	Serum creatinine: 2-2.9 times the lowest previous value; Urine output: <0.5 ml/kg/h for >=12 h	Serum creatinine: >=3 times the lowest previous value OR >=2.5 absolute value; Urine output: <0.3 ml/kg/h for >=24 h or anuria for >=12 h OR need of dialysis	Death
71.	**Nephrolithiasis/Nephrocalcinosis:** A disorder characterized by the presence of calcification or stone in the renal system	-	Asymptomatic and NOT needing any treatment	Leading to impaired renal function or renal dilatation and needing treatment	-	-
72.	**Urinary Tract Infection (UTI):** A disorder of the urinary tract caused by micro-organisms	-	UTI NOT associated with complications such as pyelonephritis, or prolongation of hospital stay or requiring new hospital admission	UTI necessitating admission to hospital or prolongation of hospital stay	-	-
Miscellaneous						
73.	**Neonatal Edema** (Definition C154931 | 10054497): Accumulation of an excessive amount of fluid in cells or intercellular tissues.	Mild edema; no change in age-appropriate behavior; no care changes indicated	Moderate edema; no change in age-appropriate behavior; requiring minor care changes (e.g. alteration in fluid management)	Severe edema; limiting age-appropriate behavior; requiring major care changes (e.g. diuretics)	Life-threatening consequences (e.g. respiratory failure, shock); requiring urgent major care changes (e.g. intubation, dialysis)	Death
74.	**Gangrene:** A disorder characterized by necrosis of part of the body	-	Gangrene of distal phalanx	Gangrene extending > 1 phalanx of any digit or metacarpals/metatarsals	Gangrene including long bones of limbs	-
75.	**Osteopenia:** A disorder characterized by reduction in body mass or density	Asymptomatic, diagnosed on imaging studies	Causing fracture or prolongation of respiratory support	-	-	-
76.	**Accidental Injury:** A disorder characterized by injury to the body due to a fall or violent contact with an external object, including accidental asphyxia	Minor external bleeding or injury	Need for minor surgery	Fracture, need for major surgery or major bleeding including intracranial bleeding.	Accompanied by cardiorespiratory compromise	Death
77.	**Inguinal hernia:** A disorder characterized by herniation of abdominal contents into the inguinal canal	-	Reducible hernia, unobstructed	Obstructed	Strangulated	-

S.No	Adverse Events	Grade 1 (Mild)	Grade 2 (Moderate)	Grade 3 (Severe)	Grade 4 (Life-threatening)	Grade 5 (Death)
78.	**Pressure sore:** A disorder characterized by formation of ulcers over pressure sites	-	Small (< 2 cm)- healing happens in <1 week.	Major pressure sore (>2 cm) and requiring surgical intervention or non-healing pressure sore	-	-
79.	**Extravasation injury:** A disorder characterized by injury caused to skin due to extravasation of intravenous fluid	Swelling of extremity	Superficial skin necrosis	Deep skin necrosis	-	-
Any other AE						
80.	**Any other AE**	Mild; asymptomatic or mild symptoms; clinical or diagnostic observations only; no change in baseline age-appropriate behavior*; no change in baseline care or monitoring indicated	Moderate; resulting in minor changes of baseline age-appropriate behavior*; requiring minor changes in baseline care or monitoring***	Severe; resulting in major changes of baseline age-appropriate behavior* or non-life-threatening changes in basal physiological processes**; requiring major change in baseline care or monitoring****	Life-threatening; Resulting in life-threatening changes in basal physiological processes**; requiring urgent major change in baseline care	Death related to AE